

VIRTUALWORKS LAB INTERNSHIP

PHARMACOVIGILANCE

TASK 2

ADR REPORTING & DOCUMENTATION

Structured ADR Reporting Form Based on a Case Scenario

Submitted by	Hemraj Sonawane
Course / Program	VirtualWorks Lab Internship Pharmacovigilance
Date	29 August 2026

Note: This report uses a fictional patient case created only for internship learning and documentation practice. It is not an actual patient safety report.

1. Aim

To complete a structured adverse drug reaction (ADR) report from a patient case scenario and document the suspected reaction in a format similar to pharmacovigilance reporting practice.

2. Introduction

ADR reporting is an important part of pharmacovigilance because it helps collect safety information after medicines are used in patients. A useful ADR report should clearly describe the patient, suspected reaction, suspected medicine, time relationship, treatment given, outcome and any other factors that may explain the event. In India, the Pharmacovigilance Programme of India (PvPI) uses a suspected ADR reporting form for voluntary reports from healthcare professionals. This internship task follows the same basic data elements in a simplified academic format.

3. Case Scenario

A 34-year-old male patient (initials R.K.), weighing approximately 68 kg, visited an outpatient clinic with symptoms of acute bacterial sinusitis. He was prescribed amoxicillin + clavulanic acid 625 mg orally three times daily from 24 August 2026. Paracetamol 500 mg was also advised only when required for headache and fever.

On 26 August 2026, after about two days of antibiotic therapy, the patient developed a red maculopapular rash over the chest and both arms with itching. There was no breathing difficulty, facial swelling, mucosal involvement or loss of consciousness. The antibiotic was stopped on the same day and cetirizine 10 mg once daily was given for symptomatic relief. The rash and itching gradually improved and had almost completely resolved by 28 August 2026. The patient had no known previous drug allergy and no recent exposure to a new food, cosmetic or other medicine. The suspected medicine was not restarted.

4. Initial ADR Identification

Suspected ADR	Maculopapular skin rash with pruritus
Suspected drug	Amoxicillin + clavulanic acid
Time to onset	Approximately 2 days after starting treatment
Seriousness	Non-serious; no hospitalization or life-threatening features
Outcome	Recovered / recovering after withdrawal and symptomatic treatment



SUSPECTED ADVERSE DRUG REACTION REPORTING FORM

For **VOLUNTARY** reporting of ADRs by Healthcare Professionals

INDIAN PHARMACOPOEIA COMMISSION (National Coordination Centre-Pharmacovigilance Programme of India)

Ministry of Health & Family Welfare, Government of India, Sector-23, Raj Nagar, Ghaziabad-201002

PvPI Helpline (Toll Free) : 1800-180-3024 (9:00 AM to 5:30 PM, Monday-Friday)

Initial Case ☐		Follow-up Case ☐		FOR AMC / NCC USE ONLY							
A. PATIENT INFORMATION *				Reg. No. / IPD No. / OPD No. / CR No. :							
1. Patient Initials: R.K.		2. Age or date of birth: 34 years		AMC Report No. :							
3. Gender: M ☐ F ☐ Other ☐		4. Weight (in Kg.) 68 kg		Worldwide Unique No. :							
B. SUSPECTED ADVERSE REACTION *				12. Relevant investigations with dates : Clinical assessment only. No specific laboratory investigation was required for this mild cutaneous reaction. No systemic features were observed.							
5. Event / Reaction start date (dd/mm/yyyy)		26/08/2026		13. Relevant medical / medication history (e.g. allergies, pregnancy, addiction, hepatic, renal dysfunction etc.) No known previous drug allergy or major chronic illness. No recent new food or cosmetic exposure. Paracetamol 500 mg was advised only as needed for headache or fever.							
6. Event / Reaction stop date (dd/mm/yyyy)		28/08/2026									
7. Describe Event/Reaction management with details, if any Red maculopapular rash over the chest and both arms with itching, beginning about 2 days after starting amoxicillin + clavulanic acid. There was no breathing difficulty, facial swelling, mucosal involvement or loss of consciousness. The suspected antibiotic was stopped on 26/08/2026. Cetirizine 10 mg once daily was given for symptomatic relief. The rash and itching improved and were nearly resolved by 28/08/2026.				14. Seriousness of the reaction : No ☐ if Yes ☐ (please tick anyone) ☐ Death (dd/mm/yyyy) ☐ Congenital-anomaly ☐ Life threatening ☐ Disability ☐ Hospitalization-Initial/Prolonged ☐ Other Medically important							
				15. Outcome: ☐ Recovered ☐ Recovering ☐ Not Recovered ☐ Fatal ☐ Recovered with sequelae ☐ Unknown							
C. SUSPECTED MEDICATION(S) *											
S. No.	8. Name (Brand / Generic)	Manufacturer (if known)	Batch No. / Lot No.	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates		Indication	Causality Assessment
								Date Started	Date Stopped		
i	Amoxicillin + clavulanate				625 mg	Oral	TID	24/08/26	26/08/26	Sinusitis	Probable / Likely
ii											
iii											
iv [#]											
9. Action taken after reaction (please tick)								10. Reaction reappeared after reintroduction of suspected medication (please tick)			
S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown	Yes	No	Effect unknown	Dose (if re-introduced)	
i	X								X		
ii											
iii											
iv											
11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)											
S. No.	Name (Brand / Generic)	Dose	Route	Frequency (OD, BD, etc.)	Therapy Dates		Indication				
					Date Started	Date Stopped					
i	Paracetamol	500 mg	Oral	PRN	24/08/26	26/08/26	Headache / fever				
ii											
iii [#]											
Additional Information :						D. REPORTER DETAILS *					
Academic internship simulation only - not for submission to PvPI. Suspected medicine was not reintroduced. Positive dechallenge was observed. WHO-UMC causality: Probable / Likely.						16. Name & Address : <u>Hemraj Sonawane</u>					
						<u>VirtualWorks Lab Internship - Pharmacovigilance</u>					
						Pin : <u>N/A</u> Email : <u>Not provided</u>					
						Contact No- : <u>Not provided</u>					
						Occupation : <u>Student Intern - Pharmacovigilance</u> Signature : <u>Hemraj Sonawane</u>					
						17. Date of this report (dd/mm/yyyy) : 29/08/2026					
Signature and Name of Receiving Personnel :											
Confidentiality : The patient's identity is held in strict confidence and protected to the fullest extent. Submission of a report does not constitute an admission that medical personnel or manufacturer or the product caused or contributed to the reaction. Submission of an ADR report does not have any legal implication on the reporter.											

[#] Use separate page for more information

* Mandatory Fields for suspected ADR Reporting Form

ADVICE ABOUT REPORTING

A. What to report?

All adverse events should be reported

Report non-serious, known or unknown, frequent or rare adverse drug reactions due to Medicines, Vaccines & Herbal Products.

Report every serious adverse drug reactions. A reaction is serious when the patient outcome is :

- Death
- Life-threatening
- Hospitalization (initial or prolonged)
- Disability (significant, persistent or permanent)
- Congenital anomaly
- Report intervention to prevent permanent impairment or damage

NOTE : Serious/Adverse Event following immunization can also be reported in Serious AEFI case Notification Form available on <http://www.ipc.gov.in>

B. Who can report?

All healthcare professionals (Clinicians, Dentists, Pharmacists and Nurse etc.) can report adverse drug reactions

C. Where to report?

Duly filled in Suspected Adverse Drug Reaction Reporting Form can be sent to the nearest Adverse Drug Reaction Monitoring Centre (AMC) or directly to the National Coordination Centre (NCC) for PvPI.

Call on Helpline (Toll Free) 1800 180 3024 to report ADRs or directly mail this filled form to pvpi.ipc@gov.in

A list of nationwide AMCs is available at : <http://www.ipc.gov.in>, http://www.ipc.gov.in/PvPI/pv_home.html

D.What happens to the submitted information?

- Information provided in this form is handled in strict confidence. The causality assessment is carried out at AMCs by using WHO-UMC scale. The analyzed forms are forwarded to the NCC-PvPI through ADR database. Finally the data is analyzed and forwarded to the Global Pharmacovigilance Database managed by WHO Uppsala Monitoring Centre in Sweden.
- The reports are periodically reviewed by the NCC-PvPI. The information generated on the basis of these reports helps in continuous assessment of the benefit-risk ratio of medicines.
- The Signal Review Panel of PvPI reviews the data and suggests any interventions that may be required.

E. Mandatory fields for suspected ADR Reporting Form (*)

Patient initials, age at onset of reaction, reaction term(s), date of onset of reaction, suspected medication(s) & reporter information.

For Adverse Drug Reaction Reporting Tools

- E-mail : pvpi.ipc@gov.in
- PvPI Helpline (Toll Free) : 1800 180 3024 (9:00 AM to 5:30 PM, Monday-Friday)
- ADR Mobile App : "ADRPvPI"

6. Relevant Medical History and Investigations

- No known previous drug allergy reported.
- No major chronic illness mentioned in the case scenario.
- No breathing difficulty, facial oedema or mucosal involvement was observed.
- No specific laboratory investigation was required for this mild cutaneous reaction in the simulated case.
- No alternative recent medicine, new food or cosmetic exposure was identified from the available history.

7. Causality Assessment

WHO-UMC category selected: Probable / Likely

Reasoning:

- The rash appeared after the suspected antibiotic was started, giving a reasonable time relationship.
- Skin rash and hypersensitivity reactions are recognized adverse effects of penicillin-class antibiotics.
- The reaction improved after the suspected medicine was stopped (positive dechallenge).
- No strong alternative cause was identified from the information available.
- Rechallenge was not performed, which is appropriate because intentional re-exposure is not necessary for a mild suspected hypersensitivity reaction.

8. Assessment of Seriousness

This case is classified as non-serious because it did not result in death, hospitalization, prolonged hospitalization, disability, congenital anomaly or an immediately life-threatening condition. The patient was managed by stopping the suspected drug and giving symptomatic treatment.

9. Reporter Details - Internship Simulation

Reporter name	Hemraj Sonawane
Role	Student Intern - Pharmacovigilance
Program	VirtualWorks Lab Internship Pharmacovigilance
Report date	29 August 2026
Contact / Signature	Not applicable - academic simulation

10. My Documentation Notes

While preparing this report, I first checked whether the event occurred after the medicine was started. I then compared the suspected drug with the patient's other medicine and looked for another possible reason for the rash. The improvement after stopping amoxicillin + clavulanic acid supported the suspicion. I documented the case as non-serious and selected a probable/likely relationship because the timing and dechallenge were supportive, but there was no rechallenge.

11. Final ADR Case Summary

Patient	R.K., 34-year-old male
Suspected reaction	Maculopapular rash with pruritus
Suspected drug	Amoxicillin + clavulanic acid 625 mg
Indication	Acute bacterial sinusitis
Onset	Approximately 2 days after starting therapy
Action taken	Suspected drug discontinued
Outcome	Recovered / nearly resolved
Seriousness	Non-serious
Causality	Probable / Likely (WHO-UMC)

12. Conclusion

The case was documented as a suspected non-serious cutaneous ADR related to amoxicillin + clavulanic acid. The close time relationship, known association of the drug with skin reactions and improvement after withdrawal support a probable/likely causal relationship. This task helped demonstrate how patient details, medicine history, reaction chronology, action taken, outcome and causality are recorded in a structured pharmacovigilance report.

13. References

1. Central Drugs Standard Control Organization (CDSCO) / Pharmacovigilance Programme of India. Suspected Adverse Drug Reaction Reporting Form, Version 1.4.
2. Indian Pharmacopoeia Commission. Pharmacovigilance Programme of India - ADR Reporting Forms.
3. World Health Organization - Uppsala Monitoring Centre. The use of the WHO-UMC system for standardized case causality assessment.
4. British National Formulary / standard prescribing references: adverse effects of amoxicillin and clavulanic acid (general drug-safety reference).

Academic integrity note: The patient scenario is fictional and has been prepared for learning purposes only. No real patient identifiers or confidential health information are included.